

Documentation-Driven Threat Analysis

DermaTriage Governed DDTA Documentation

A5 Candidate R1 - Pending A6-A12

Current research artifact

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1 DermaTriage - Governed DDTA Documentation Candidate

Status: A5 CLOSED CANDIDATE / NOT YET PROMOTED

Coverage: project framing through FunctionalRequirement cumulative D3 closure

Pending before final documentation promotion: A6-A12

Methodology authority: DDTA_DOCUMENTATION_BA_AUTHORING_GUIDE_R4

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This file is the **clean project-document candidate**. Research commentary and methodological reasoning are intentionally excluded. Source-supported implementation/configuration evidence is retained only in clearly marked binding/evidence sections. A6-A12 may still refine split, specialization, security, terminology, bindings and promotion readiness.

1.1 1. Project problem framing

DermaTriage addresses the problem of supporting early triage of dermatological cases concerning potentially oncological skin lesions, using the information available for the case to determine a priority of care and support routing toward appropriate specialist assistance.

1.1.1 Scope boundary

Governed in the current candidate:

- urgency/priority-oriented triage behavior;
- specialist-routing responsibility at macro level;
- management of clinical review results;
- controlled adaptation using accumulated clinical review evidence.

Not established as governed project authority:

- authority for definitive clinical diagnosis;
- complete specialist-selection/booking responsibility;
- automatic production-deployment authority after adaptation qualification;
- complete semantics of every source-level configuration value.

1.2 2. Canonical macro view

DERMATriage

```
|
|-- MR-01  Valutazione di triage del caso dermatologico
|
|-- MR-02  Indirizzamento specialistico
|
|-- MR-03  Gestione della validazione clinica degli esiti
|
`-- MR-04  Adattamento controllato sulla base della revisione clinica
           dependsOn MR-03
```

Stable IDs are identities, not reading order. The narrative projection above is for human readability.

2 3. MR-01 - Valutazione di triage del caso dermatologico

2.1 Intent

Determinare, dalle informazioni disponibili sul caso dermatologico, una valutazione di urgenza utile a stabilire la priorit  di presa in carico specialistica.

2.2 Context and scope

The responsibility concerns urgency/priority evaluation for a dermatological case. It does not establish authority for definitive clinical diagnosis and does not own the complete specialist-routing responsibility.

2.3 DEC-01 - Continuit  del triage in assenza di immagine

Decision kind: SELECTABLE

Status: PASS

DermaTriage permits urgency evaluation to continue when no image is available, using available symptom information rather than treating image availability as an absolute precondition for triage.

2.3.1 FR-01 - Determinazione dell'urgenza su base sintomatologica in assenza di immagine

Parent: DEC-01

Status: PASS

Normative clause

Quando per il caso dermatologico non   disponibile un'immagine, DermaTriage MUST determinare l'urgenza sulla base delle informazioni sintomatologiche disponibili per il caso.

Source examples such as itching, bleeding, growing, changing and pain illustrate current symptom evidence and are not treated here as a closed normative vocabulary.

2.3.2 Preserved gap

GAP-DERMA-NOIMAGE-INPUT-01 - the complete required/optional symptom input contract and missing-value semantics are not specified.

2.4 DEC-02 - Adozione della P-scale per la priorit  operativa

Decision kind: SELECTABLE

Status: PASS

DermaTriage represents operational triage priority using P1-P4, derived from the urgency evaluation of the case.

2.4.1 FR-02 - Derivazione della priorit  P-scale dalla valutazione di urgenza

Parent: DEC-02

Status: PASS

Normative clauses

HIGH + confidence > 0.85	-> P1
HIGH	-> P2
MEDIUM	-> P3
LOW	-> P4

The strict comparator > is preserved.

2.4.2 Preserved gaps

- GAP-DERMA-PMAP-INPUT-01 - complete semantics of the urgency/confidence inputs, including missing/invalid values, are not specified.
- GAP-DERMA-NOIMAGE-PMAP-BINDING-01 - the documentation does not establish that the urgency produced by FR-01 necessarily provides every input required by FR-02.
- GAP-DERMA-SLA-01 - source values 24h/48h/72h/7 days are preserved as evidence, but trigger event, owned outcome, responsibility and whether the values are limits/targets/recommendations are not sufficiently governed.
- GAP-DERMA-PATH-01 - predicted_pathology is source-observed but its governed obligation status and diagnostic-authority boundary remain not established.

2.5 MR-01 cumulative D3 disposition

PASS. FR-01 and FR-02 provide the independently assessable operational obligations currently justified under DEC-01 and DEC-02. Remaining source meanings are preserved as gaps or routed evidence rather than invented requirements.

3 4. MR-02 - Indirizzamento specialistico

3.1 Intent

Determinare un'indicazione di destinazione specialistica per il caso dermatologico, al fine di supportarne il successivo instradamento verso l'assistenza appropriata.

3.2 Status

STOP AT MR.

The source supports that DermaTriage includes specialist-oriented output/indication, but does not sufficiently govern:

- specialist-domain vocabulary;
- selection rule;
- required inputs;
- fallback behavior;
- actual assignment/booking responsibility;
- complete relationship with P-scale priority.

No Decision or FR is created merely to fill the hierarchy.

4 5. MR-03 - Gestione della validazione clinica degli esiti

4.1 Intent

Supportare la registrazione e l'utilizzo della validazione o correzione effettuata da un professionista sanitario sugli esiti prodotti da DermaTriage, mantenendo distinguibile il risultato della revisione dall'esito originario.

4.2 Responsibility boundary

The healthcare professional owns the clinical judgment. DermaTriage owns management, recording and correlation of the review result.

4.3 DEC-03 - Separazione tra esito originario e revisione clinica

Decision kind: SELECTABLE

Status: PASS

DermaTriage keeps the original system outcome distinguishable from the subsequent clinical review result.

4.3.1 FR-03 - Registrazione correlata della revisione clinica

Parent: DEC-03

Status: PASS

NC-03.1

When a clinical validation/correction is supplied for a DermaTriage outcome, DermaTriage MUST record the review result associated with the original outcome.

NC-03.2

The recorded review result MUST distinguish confirmation from correction.

4.3.2 Preserved gaps

- GAP-DERMA-REVIEW-CONTENT-01 - the complete correction/review content model is not specified.
- GAP-DERMA-REVIEW-LIFECYCLE-01 - overwrite/history/retention/finality semantics are not specified.
- GAP-DERMA-REVIEW-DISAGREEMENT-BINDING-01 - the normative relationship among correction, disagreement and the source-observed agrees state is not fully specified.

4.4 MR-03 cumulative D3 disposition

PASS.

5 6. MR-04 - Adattamento controllato sulla base della revisione clinica

5.1 Intent

Consentire l'evoluzione controllata del comportamento di DermaTriage utilizzando evidenza derivata dalle validazioni e correzioni cliniche accumulate, evitando che modifiche degradanti vengano accettate senza controllo.

5.2 Dependency

MR-04 dependsOn MR-03 = PASS because adaptation consumes clinical-review meaning owned by MR-03.

5.3 Decision family

DEC-04 Accumulation-gated adaptation
DEC-05 Separate adaptation paths
DEC-06 Comparative acceptance
DEC-07 Asymmetric quality prioritization
DEC-08 Classification adaptation reversibility
DEC-09 Bounded recent evidence - prompt path
DEC-10 Disagreement as qualifying classifier evidence
DEC-11 Corrected P-scale as supervision source

5.4 DEC-04 - Attivazione dell'adattamento su accumulo di evidence clinica

DermaTriage uses a non-immediate adaptation strategy: each adaptation process is activated only when relevant accumulated clinical evidence satisfies a governed accumulation condition for that process.

5.4.1 FR-04 - Attivazione del percorso di evoluzione del prompt

Parent: DEC-04

Status: PASS

Quando l'evidence di correzione clinica pertinente accumulata raggiunge la soglia prevista per il percorso di evoluzione del prompt, DermaTriage MUST attivare un ciclo di evoluzione del prompt.

Semantic parameter: PromptEvolutionThreshold=N

Current documented binding: N=10

Exact fixed normative status: NOT SPECIFIED

Change authority: NOT SPECIFIED

5.4.2 FR-05 - Attivazione del percorso di adattamento classificatorio su accumulo di evidence di disaccordo

Parent: DEC-04

Status: PASS

Quando l'evidence qualificante accumulata raggiunge la soglia prevista per il percorso di adattamento classificatorio, DermaTriage MUST attivare il relativo ciclo di adattamento.

Semantic parameter: ClassifierAdaptationThreshold=N

Current documented binding: N=50

Exact fixed normative status: NOT SPECIFIED

Change authority: NOT SPECIFIED

5.4.3 Preserved gaps under DEC-04

- GAP-DERMA-ADAPT-COUNTING-01 - reset, deduplication, reuse, persistence, multiple-review and concurrency counting semantics are not specified.
- GAP-DERMA-ADAPT-THRESHOLD-AUTH-01 - governance/change authority of concrete threshold values is not specified.

5.5 DEC-05 - Separazione dei percorsi di adattamento per capability

DermaTriage governs adaptation through distinct lifecycle paths for different behavioral capabilities. Conditions or outcomes belonging to one path do not automatically satisfy those of another path.

5.5.1 FR-11 - Indipendenza dei lifecycle di adattamento

Parent: DEC-05

Status: PASS / STOP

NC-11.1

The satisfaction of an activation condition for one adaptation path **MUST NOT**, by itself, be treated as satisfaction of the activation condition of another path.

NC-11.2

Evidence qualifying for one adaptation path **MUST NOT**, by itself, be treated as qualifying for another path.

NC-11.3

A qualification/progression result reached by one adaptation path **MUST NOT**, by itself, be treated as an equivalent lifecycle result for another path.

Shared data may be reused if it independently satisfies the governed rules of each path; shared data does not imply shared lifecycle semantics.

5.6 DEC-09 - Uso di evidence recente e limitata nel percorso di evoluzione del prompt

DermaTriage uses bounded recent clinical-correction evidence rather than unrestricted history for prompt evolution.

5.6.1 FR-06 - Selezione dell'evidence recente per l'evoluzione del prompt

Parent: DEC-09

Status: PASS / STOP

Per ciascun ciclo di evoluzione del prompt, DermaTriage **MUST** costruire l'evidence set utilizzando una finestra scorrevole e limitata delle correzioni cliniche piu' recenti pertinenti al percorso di evoluzione del prompt.

Semantic parameter: PromptEvidenceWindowSize=N

Current documented binding: N=20

Exact fixed normative status: NOT SPECIFIED

Change authority: NOT SPECIFIED

Preserved gap: GAP-DERMA-PROMPT-WINDOW-01 - ordering, membership, underfill, deduplication, reuse/overlap and exact scope semantics are not specified.

5.7 DEC-10 - Qualificazione del disaccordo clinico come evidence per l'adattamento classificatorio

For urgency-classification adaptation, DermaTriage treats clinical reviews expressing disagreement with the relevant AI outcome as qualifying evidence on the disagreement dimension.

5.7.1 FR-07 - Qualificazione del disaccordo clinico come evidence per l'adattamento classificatorio

Parent: DEC-10

Status: PASS / STOP

Nel determinare l'evidence clinica qualificante per il percorso di adattamento della classificazione di urgenza, DermaTriage MUST considerare qualificanti soltanto le revisioni cliniche che esprimono disaccordo rispetto all'esito AI pertinente.

Governed concept: ClinicianDisagreement

Source-observed current encoding: agrees == False

The encoding is realization/data-state evidence, not the normative semantic concept.

Confirmed consumption: FR-07 -> FR-05.

5.8 DEC-11 - Derivazione della supervision classificatoria dalla priorita' clinicamente corretta

For urgency-classification adaptation, DermaTriage uses the clinically corrected P-scale priority as the governed source from which to derive the classifier-supervision target.

5.8.1 FR-08 - Derivazione del target di supervisione dalla priorita' P-scale corretta

Parent: DEC-11

Status: PASS / STOP

For classifier-adaptation cases with a clinically corrected P-scale priority, DermaTriage MUST derive the supervision target according to:

P1 -> HIGH

P2 -> HIGH

P3 -> MEDIUM

P4 -> LOW

The transformation is governed semantic meaning, not a replaceable N parameter.

Preserved gap: GAP-DERMA-SUPERVISION-INPUT-01 - missing, invalid, out-of-domain or conflicting corrected-priority behavior is not specified.

5.9 DEC-06 - Accettazione comparativa dell'adattamento della classificazione di urgenza

A candidate urgency-classification adaptation is not adopted merely because it has been produced; it must first pass comparative evaluation against the applicable reference under governed acceptance criteria.

5.9.1 FR-09 - Qualificazione comparativa del candidato di adattamento classificatorio

Parent: DEC-06

Status: PASS / STOP

NC-09.1

When a candidate urgency-classification adaptation is considered for adoption, DermaTriage MUST evaluate it comparatively against the applicable reference version using governed acceptance criteria.

NC-09.2

The candidate MUST be considered qualified for adoption only if all applicable governed comparative acceptance criteria are satisfied.

qualified for adoption does not mean automatically deployed.

Preserved gap: GAP-DERMA-DEPLOY-01 - final promotion authority and automaticity after qualification are not specified.

5.10 DEC-07 - Prioritizzazione asimmetrica delle metriche di qualita'

DermaTriage governs an asymmetric acceptance policy in which sensitivity and false-low performance may not worsen relative to the reference, while overall accuracy may degrade only within a bounded tolerance.

5.10.1 A5 disposition

NO NEW FR / ROUTE TO A7.

This Decision governs additional quality properties of FR-09 rather than a new autonomous operational capability.

Current specialization candidates to preserve for A7:

SensitivityNonDegradation

FalseLowNonDegradation

LimitedAccuracyDegradation

Semantic parameter for the third property: AccuracyDegradationTolerance=T

Current textual binding: T=5%

Absolute-versus-relative interpretation: NOT SPECIFIED

Exact reference/evaluation population/change authority: NOT SPECIFIED

Concrete non-security subtype design is outside the current thesis specialization scope unless later explicitly reopened.

Preserved gap: GAP-DERMA-ACCEPT-BINDING-01.

5.11 DEC-08 - Reversibilita' dell'adattamento della classificazione in caso di degrado

An already adopted urgency-classification adaptation that manifests material degradation according to governed post-adoption criteria must be reversible by restoring a previous acceptable version or state.

5.11.1 FR-10 - Revoca di un adattamento classificatorio degradante

Parent: DEC-08

Status: PASS / STOP

Per un adattamento della classificazione di urgenza già adottato che manifesti degrado materiale secondo i criteri post-adoption governati, DermaTriage MUST supportarne la revoca mediante il ripristino di una versione o stato precedente accettabile.

The requirement does not establish that rollback must be automatic.

Semantic parameter: RollbackAccuracyDegradationThreshold=R

Current textual binding: R=5%

R is not assumed identical to pre-adoption T merely because the current literals are equal.

Preserved gap: GAP-DERMA-ROLLBACK-BINDING-01 - unit, reference, population, observation window, timing, automaticity, authorization and exact rollback target remain insufficiently governed.

5.12 MR-04 cumulative D3 disposition

PASS / CLOSED at A5.

No MR-04 Decision remains without an A5 disposition:

Decision	A5 result
DEC-04	FR-04 + FR-05 PASS
DEC-05	FR-11 PASS / STOP
DEC-06	FR-09 PASS / STOP
DEC-07	no FR; route to A7
DEC-08	FR-10 PASS / STOP
DEC-09	FR-06 PASS / STOP
DEC-10	FR-07 PASS / STOP
DEC-11	FR-08 PASS / STOP

6 7. Cross-requirement and cross-MR relations

Confirmed:

MR-04 dependsOn MR-03

FR-07 -> FR-05 [qualifying disagreement evidence is accumulated by FR-05]

Not asserted without additional evidence:

FR-01 -> FR-02

FR-07 -> FR-08

FR-08 -> FR-09

FR-09 -> FR-10

MR-04 dependsOn MR-01

MR-04 may reference/consume the P-scale domain governed by MR-01 without acquiring a second parent or automatically creating a macro dependency.

7 8. Parameter and binding registry - current candidate

Semantic role	DDTA semantic form	Current source
prompt adaptation accumulation threshold	PromptEvolutionThreshold=N	
classifier adaptation accumulation threshold	ClassifierAdaptationThreshold=N	
prompt recent-evidence window size	PromptEvidenceWindowSize=N	
pre-adoption accuracy degradation tolerance	AccuracyDegradationTolerance=T	
post-adoption rollback accuracy degradation threshold	RollbackAccuracyDegradationThreshold=R	

T == R is not established.

8 9. Information/data-state bridge ledger - current candidate

Governed semantic concept/reference	Source-observed/current binding
ClinicianDisagreement	agrees == False
ClinicallyCorrectedOperationalTriagePriority	source review/correction data carrying P1-P4
ClassifierUrgencyTarget	HIGH/MEDIUM/LOW; possible enum/string/index realization
PromptEvolutionThreshold	current 10
ClassifierAdaptationThreshold	current 50
PromptEvidenceWindowSize	current 20

9 10. Preserved project gap register

Gap	Area	Preserved meaning
Diagnostic authority boundary	framing / MR-01	definitive clinical diagnosis authority not established
GAP-DERMA-NOIMAGE-INPUT-01	FR-01	complete symptom input contract not specified
GAP-DERMA-PMAP-INPUT-01	FR-02	complete urgency/confidence input semantics not specified
GAP-DERMA-NOIMAGE-PMAP-BINDING-01	MR-01	no-image urgency -> P-scale input binding not specified
GAP-DERMA-SLA-01	MR-01 downstream	SLA trigger/outcome/owner semantics not established
GAP-DERMA-PATH-01	MR-01 downstream	pathology output obligation/authority not established
specialist selection semantics	MR-02	domain/rules/input/fallback/booking not specified
GAP-DERMA-REVIEW-CONTENT-01	FR-03	complete review payload not specified
GAP-DERMA-REVIEW-LIFECYCLE-01	FR-03	overwrite/history/retention/finality not specified
GAP-DERMA-REVIEW-DISAGREEMENT-BINDING-01	MR-03/MR-04	correction/disagreement/agrees relation not specified
GAP-DERMA-ADAPT-COUNTING-01	FR-04/05	counting/reset/dedup/reuse semantics not specified
GAP-DERMA-ADAPT-THRESHOLD-AUTH-01	FR-04/05	threshold change authority not specified
GAP-DERMA-PROMPT-WINDOW-01	FR-06	exact window membership/lifecycle semantics not specified
GAP-DERMA-SUPERVISION-INPUT-01	FR-08	invalid/missing corrected-priority behavior not specified
GAP-DERMA-DEPLOY-01	FR-09 lifecycle	final deployment authority/automaticity not specified
GAP-DERMA-ACCEPT-BINDING-01	DEC-06/07	complete quantitative acceptance binding not specified
GAP-DERMA-ROLLBACK-BINDING-01	DEC-08	complete rollback trigger binding not specified

10 11. Source-observed evidence pending A7/A8/A10 review

The following source facts are preserved but **not yet promoted** to governed Specialized/Security requirements or stable realization bindings by this candidate:

10.1 11.1 Non-security quality specialization evidence - pending A7

- sensitivity non-degradation relative to the applicable reference;
- false-low non-degradation relative to the applicable reference;
- bounded accuracy degradation;
- source documents report a current 5% literal for accuracy tolerance.

10.2 11.2 Security/privacy evidence - pending A8

Source documents currently describe:

- X-API-Key on protected DermaTriage administrative/B4-integrated operations;
- B4 JWT authentication;
- anonymized research patient identifiers using a PAT_XXXX style;
- no raw patient images stored in the RAG CSV;
- B4 patient uploads processed in memory and not stored permanently in the research dataset.

These observations are **A8 candidate evidence**, not yet final SecurityRequirements. Mechanisms such as X-API-Key/JWT are not automatically the security property.

10.3 11.3 Realization/configuration evidence - pending A10

Examples include:

- current named models and four-stage pipeline;
- endpoint paths;
- environment variables;
- prompt storage path;
- database filenames;
- fine-tuning layers, learning rates, epochs and class weights;
- model version/file identifiers;
- current threshold tuning values not already promoted to governed semantics.

11 12. Current gate state

A0	Authority/source gate	PASS
A1	Project framing	PASS
A2	MacroRequirement / D1	CUMULATIVE PASS
A3	Semantic sufficiency	PASS for decomposed scope
A4	Decision / D2	CUMULATIVE PASS / CLOSED
A5	FunctionalRequirement / D3	CUMULATIVE PASS / CLOSED
A6	Requirement split	NEXT
A7	SpecializedRequirement	NOT STARTED
A8	SecurityRequirement	NOT STARTED
A9	Cross-MR downstream review	NOT STARTED
A10	Terminology/information/bindings	NOT STARTED

A11 Propagation/regression	NOT STARTED
A12 Completeness/promotion readiness	NOT STARTED
BA Accepted Base Analysis	BLOCKED

This candidate must not be promoted as the final DermaTriage governed baseline before A6-A12 complete.